

SUMMATIVE ASSESSMENT

FULL NAME & SURNAME	
ID NUMBER:	
NAME OF ASSESSOR	
DATE OF ASSESSMENT	
VENUE	

MODEL ANSWERS

	ACHIEVED	NOT ACHIEVED
KNOWLEDGE		
SKILLS		

Signature of learner

Signature of Assessor

ASSESSMENT PACK

Please complete the following sections (A and B) before commencing with this assessment. The moderator of this assessment will complete section C.

Section A Learner Information															
Name:															
Surname:															
Date:															
Contact telephone no:															
Learnership agreement no:															
Company:										Site:					
ID															

Section B Assessor Information															
Name:															
Surname:															
Date:															
Contact telephone no:															
Assessor no:															
Provider no:										Site:					
ID															

Section C Moderator Information															
Name:															
Surname:															
Date:															
Contact telephone no:															
Moderator no:															
Company:										Site:					
ID															

Results:

P A R T

1: Assessors Guide

1. Instructions to Assessor

Introduction:

This assessment guide has been designed as a generic assessment guide and is intended for use by the accredited Security Industry Training Providers.

Purpose of the assessment

The purpose of summative assessment against this unit standard is to:

- ◆ Award credits to the NQF to learners who are able to Handle and defuse security related conflict

Learning assumptions

The following knowledge, skills, attitude and/or equivalent:

- ◆ A knowledge, comprehension and application of language at NQF 02.

Assessment methods

The following assessment methods will be used for the summative assessments:

- ◆ Written and/or/verbal questioning
- ◆ Product sample and on site assessment

2. Assessment Process

General

- ◆ Use the assessment guide and your latest company policies and standard operating procedures to assess the evidence received from the learner.
- ◆ Use the section: Addition Comments/Questions to note down any further comments or questions on the evidence assessed.
- ◆ Use the model answers as a guideline to assess the learner's answers to the assessment questionnaire.
- ◆ The learner can complete the assessment questionnaire orally. In this case, agree a date, time and venue.
- ◆ Provide the learner with a feedback within 10 working days of receiving the evidence.

Step 1 - Planning for the Assessment

Review this assessment guide to:

- ♦ Ensure that you understand all the requirements of the assessment in terms of evidence required to prove competence.
- ♦ Identify and prepare the learner for the assessment by:
 - ⇒ Completing the Assessment Plan with the learner to discuss and agree the details regarding the assessment.
 - ⇒ Completing the Assessment Preparation Checklist and getting the learner to sign.
- ♦ Ensure that you have familiarized yourself with the following:
 - ⇒ General knowledge of HIV
 - ⇒ Company procedures and policies regarding the support, and dealing of HIV in the workplace

Step 2: Complete the Assessment

- ♦ Collect the evidence in accordance with the methods and evidence requirements specified.
- ♦ Mark each question as correct or incorrect in the “Office Use” column.
- ♦ Record the evidence on the assessment guide and indicate “Competent”, “Not Yet Competent” or “Not Assessed” for each assessment criterion. Note down any comments at the back of the assessment guide.
- ♦ Ask the learner additional questions, if necessary, to clarify points. Record these on the guide.
- ♦ All questions must be complete as per the criteria specified.
- ♦ Answers provided must be similar to the model answers.

Step 3 - After the Assessment

- ♦ Prepare the feedback by writing comprehensive, developmental feedback after each section on the Assignment Sheets. In addition to this, you are required to write a summary overall feedback on the Assessment Guide.
- ♦ Provide the feedback to the learner in a safe, undisturbed in nature.
- ♦ Ensure that your feedback is developmental and supportive in nature.
- ♦ Advise the learner on what action to follow in the event of a “Not Yet Competent” rating.
- ♦ Advise the learner on what action to take where he/she feels the need to appeal against your decision.
- ♦ Allow the learner time to provide you with feedback relevant to the process.
- ♦ Record the learner’s feedback in the guide and ensure that it is given to the person responsible for the quality assurance of assessment tools.
- ♦ Ensure that the learner co-signs the assessment guide to indicate agreement with the feedback.

3. Assessment documentation required:

Step 1: Planning for the Assessment

- ♦ Assessment Plan
- ♦ Assessment Preparation Checklist
- ♦ Assessment Policy (including Appeals)
- ♦ Evidence Matrix
- ♦ Assessment Instruments

Step 2: Conducting the Assessment

- ♦ Assessor Guide
- ♦ Learner's workbook
- ♦ Summative assessment pack

Step 3: After the Assessment

- ♦ Assessment Comments
- ♦ Feedback Report

4. Specific Instructions

Please note that Part 3 Assessment Instruments are not included in this guide and are to be included by the assessor on an individual basis.

The actual summative assessments need to be completed and signed off by both learner and assessor. The assessor will take control of the completed assessment instruments and will file them under the tab for Assessment Evidence.

The completed assessment pack will be kept in safekeeping at the training provider for three months after endorsement by SASSETA and will then be returned to the learner.

Guidelines where an appeal is lodged

The normal appeal procedure prescribed by SASSETA and described by the provider's Quality Management System will be followed.

P A R T

2: Assessment Planning

ASSESSMENT PLAN

ASSESSMENT DETAILS					
Date of Assessment		Option 1	Option 2	Option 3	Option 4
TIME OF ASSESSMENT					
Start:			End:		
VENUE			Contact person		
LANGUAGE MEDIUM					
METHOD OF ASSESSMENT (please tick off the one to be used)					
OBSERVATION		ORAL		WRITTEN	
Simula- tion		Knowledge test		Knowledge test	
Product		Interview			

PRE-ASSESSMENT MEETING CHECKLIST

ACTION	YES/NO	COMMENTS
Set learner at ease; be friendly, polite and professional.		
Explain to the learner and agree on the following issues. 1. The unit standard that will be assessed 2. Date, time, venue and process to be followed during the assessment. 3. Summative assessment tools to be used for the assessment. 4. The assessment plan 5. Purpose of assessment		
Explain to the learner and agree on the role of all involved during the assessment process.		
Identify possible barriers and or disabilities of the learner.		
Explain the meaning and application of RPL.		
Explain, discuss and provide one complete set of the Appeals process documentation.		
Explain to the learner when final results will be available and how feedback will be provided.		
Discuss previous assessment results if applicable.		

I, _____ (initials and surname of learner), DECLARE THE FOLLOWING:

A copy of the unit standard(s) involved has been given to me prior to this meeting. I know I will be assessed against the criteria, which have been set to the applicable unit standards. The criteria have been discussed with me, and the procedures and purpose of the assessment has been clearly explained to me.

I am well aware of the venue, date and time that I will be assessed. I consider the period of time given to me to prepare myself for the assessment to be fair.

I understand clearly that I have the right to appeal against any decision made by the assessor during the assessment of the evidence provided by me, and that I have free access to the appeals procedures attached to this assessment pack. I understand that I have the right to be accompanied by another person during all procedures, and that I have free access to the Training Division of SBV'S Health and Safety Procedures- filed at the offices.

Signature of learner

Date

P A R T

3: Assessment Evidence

Assessment Instruments



The assessment instruments included in this assessment pack are all summative assessment instruments and are to be read in conjunction with the formative assessment instruments contained in the learner workbook. Both formative (workbook) and summative assessments are to be retained as part of the learner's portfolio of evidence.

A number of the assessment instruments contained in this assessment are workplace knowledge based questions.

This means that you will arrange with the learner, a time that is suitable, during which the learner will complete each questions.

QUESTION 1: Multiple choice questions. Choose one or more of the following by marking the correct option with a 

1. Human Immunodeficiency Virus (HIV) is...a)

- [A] Bacterial illness treated with antibiotics
- [B] A virus which has no cure but can be controlled with medicines
- [C] The virus that causes AIDS
- [D] Answers b and c**

2. HIV can infect which of the following groups....

- [A] Teens, the elderly, and newborns
- [B] College professors, the homeless, men, women, and all ethnic groups
- [C] Bisexuals, homosexuals, and heterosexuals
- [D] All of the above**

3. Which of the following increase your risk of HIV exposure...a)

- [A] Unprotected anal, oral, or vaginal sex
- [B] Sharing needles
- [C] Drug use) infection with another sexually transmitted disease
- [D] All of the above**

4. If I find I am infected, what should I do next...a)

- [A] Run away...there's no hope for me.
- [B] Talk to a doctor asap..earlier treatment means a healthier life.
- [C] Seek out support from family, friends, loved ones or HIV agencies.
- [D] Answers b and c are correct.**

5. HIV is spread from person to person by...a)

- [A] Shaking hands, kissing or hugging
- [B] Unprotected anal, oral or vaginal sexual contact
- [C] Sharing needles to inject recreational drugs
- [D] Answers b and c**

6. The best way to prevent HIV infection is...a)

- [A] Not to have sexual contact with anyone**
- [B] Stay away from those with HIV
- [C] Use latex condoms and don't share needles
- [D] I don't have to worry because I'm not at risk

QUESTION 2: True and false questions: Please make a right mark next to the correct answer.

Nr.	Question	True	False
1.	Acquired Immunodeficiency Syndrome (AIDS) is a recognized infectious disease in which the body's immune system is damaged and causes those with AIDS to be vulnerable to a number of serious, often fatal, opportunistic infections and cancers.	✓	
2.	Signs and symptoms of AIDS or other AIDS-related conditions may include loss of appetite, weight loss, fever, night sweats, skin rashes, diarrhea, tiredness, lack of resistance to infection or swollen lymph nodes.	✓	
3.	Casual contact with people with AIDS, or those who are HIV-infected can place others at risk for acquiring AIDS. The virus can be spread by casual contact such as hugging, handshaking, riding in the same car, touching the same papers or using the same office furniture, typewriter or telephone.		
4.	The Human Immunodeficiency Virus (HIV) is spread through sexual intercourse, anal or oral sex acts, sharing of hypodermic needles, syringes or other drug paraphernalia by infecting drug users, mother-to-child transmissions during pregnancy or childbirth, or rarely, through transfusions of HIV-infected blood or its components, or other exchanges of HIV-infected blood.		
5.	AIDS or its virus cannot be transmitted by using bathroom facilities, sinks, bath-tubs, toilets, drinking from the same water fountain, using dishes, utensils, or food handled by a person with AIDS, or from sneezing, or coughing.		
6.	There is a virus antibody test that tells if someone has been infected with the virus (i.e., has a positive test result). This does not mean this individual has AIDS. In fact, many who have HIV may feel well, show no disease symptoms, and may not come down with AIDS for many years.		
7.	There is no risk at all that a donor could acquire HIV or AIDS by giving blood.		
8.	Dogs, cats, and domestic animals are not a source of infection; but transmission by insects, such as mosquitoes, is a means by which the virus may be acquired.		
9.	There is a high risk of contracting HIV or AIDS by being or working around someone with either condition on a daily basis over a long period of time.		
10.	You will need to take special measures at the workplace to protect yourself or those you work with from infection by HIV.		

Requirements achieved	YES		NO	
Comments in support of decision				

QUESTION 3: Explain in your own words what does it mean if the term “window period” is used.

The window period is the time it takes for your body to produce HIV antibodies after you have been infected by the virus. In most people, this period is between 2 and 12 weeks. In a very small number of people, the process takes up to 6 months.

Requirements achieved	YES		NO	
Comments in support of decision				

QUESTION 4: Discuss the symptoms of HIV during the following stages:

- ♦ Latent infection
- ♦ Early HIV virus progression

♦ **Latent infection**

The next HIV AIDS symptoms stage begins when the immune system starts to break down. This is often preceded by subtle mutations in the virus, during which it becomes more aggressive in damaging white cells.

Several glands in the neck and armpits may swell and remain swollen for more than three months without any explanation. This is known as persistent generalized lymphadenopathy (PGL).

♦ **Early HIV virus progression**

As the HIV disease progresses, the person starts showing up other AIDS symptoms. A simple boil or warts may spread all over the body. The mouth may become infected by thrush (thick white coating), or may develop some other problem. Dentists are often the first to be in a position to make the diagnosis. People may develop severe shingles (painful blisters in a band of red skin), or herpes.

They may feel overwhelmingly tired all the time, have high temperatures, drenching night sweats, lose more than 10% of their body weight, and have diarrhoea lasting more than a month. No other cause is found and a blood test will usually be positive.

Some used to call this stage ARC, or AIDS related complex.

♦ Late HIV illness

The final stage is AIDS. Most of the immune system is intact and the body can deal with most infections, but one or two more unusual infections become almost impossible for the body to get rid of without medical help, usually intensive antibiotics.

These infections can be a nightmare for doctors and patients. The desperate struggle is to find the new germ, identify it, and give the right drug in huge doses to kill it.

The germ may be hiding deep in a lung requiring a tube (bronchoscope) to be put down the windpipe into the lung to get a sample. The person is sedated for this. It may be hiding in the fluid covering the brain and spinal cord, requiring a needle to be put into the spine (lumbar puncture). It may be hiding in the brain itself. It may hide in the liver or gallbladder or bowel. It can hide anywhere.

Requirements achieved	YES		NO	
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Comments in support of decision

QUESTION 5: Explain in your own words “opportunistic infections.”

People with advanced HIV infection are vulnerable to infections and malignancies that are called 'opportunistic infections' because they take advantage of the opportunity offered by a weakened immune system.

A partial list of the world's most common HIV-related opportunistic infections and diseases includes:

- Bacterial diseases such as **tuberculosis**, MAC, bacterial pneumonia and septicaemia (blood poisoning)
- Protozoal diseases such as toxoplasmosis, microsporidiosis, cryptosporidiosis, isopsoriasis and leishmaniasis
- Fungal diseases such as PCP, candidiasis, cryptococcosis and penicilliosis
- Viral diseases such as those caused by cytomegalovirus, herpes simplex and herpes zoster virus
- HIV-associated malignancies such as Kaposi's sarcoma, lymphoma and squamous cell carcinoma.

Requirements achieved	YES		NO	
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Comments in support of decision

QUESTION 6: List three ways in which you can get HIV.

- By having unprotected penetrative sex (anal and vaginal)
- By getting HIV infected blood into your blood stream.
- From an HIV positive mother to her baby before and during birth and when breastfeeding.

Requirements achieved	YES		NO	
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Comments in support of decision

QUESTION 7: List three preventative measures that one can follow to prevent getting infected with the HIV virus.

- **Safer Sex**
- **Use clean needles**
- **Abstaining from sex and needle**

Requirements achieved	YES		NO	
Comments in support of decision				

QUESTION 8: List the risk categories , provide a explanation of each with at least three practical examples to motivate your understanding of the risk categories.

No Risk

The following sexual behaviour involves no risk and can be regarded as safe:

- Abstinence
- Erotic massage
- Hugging
- Body rubbing
- Kissing
- Petting
- Showering or bathing together
- Masturbation
- Mutual masturbation - providing there is no contact between broken skin and semen or vaginal fluids
- Thigh sex
- Sexual fantasies
- Phone sex
- Using your own personal sex toys

Low Risk

The following sexual behaviour is regarded as safe, but is still associated with a low risk of HIV/Aids:

- Oral sex on a man (fellatio) who is wearing a condom.
- Oral sex on a woman (cunnilingus) with a latex barrier.
- Anilingus (oral-anal sex) with a latex barrier.
- Contact with urine ("golden showers" or "water sports" on unbroken skin).

If you participated in one of these acts, you are still practicing "safer" sex, but with no absolute guarantee of zero risk. Opt for an HIV-test to put your mind at rest, and try to avoid "Some Risk", "High Risk" and "Very High Risk" behaviors.

Some risk

These sexual acts are associated with some risk of HIV/Aids. They can be regarded as "fairly safe", but not without some risk:

- Oral sex (on a man or woman) without a condom or latex barrier.
- Vaginal penetration with a condom.

Anal penetrative sex with a condom. It is safer to withdraw before ejaculation. Is this an accurate description of your sex life? Then you are at some risk of contracting HIV/Aids. Opt for an HIV-test to put your mind at rest, and try to avoid "High Risk" and "Very High Risk" behaviour.

High risk

Unfortunately, many sexual practices are associated with a high risk of HIV/Aids. These include:

- Vaginal penetration without a condom
- Swallowing semen
- Sharing uncovered sex toys
- Vaginal or oral penetrative sex with a condom if using a petroleum-based lubricant
- Unprotected oral contact if blood is present
- Unprotected manual-anal intercourse (fisting) without a latex glove
- Unprotected manual-vaginal intercourse (fisting) without a latex glove
- Contact with menstrual blood

Very High Risk

There is sexual behaviour associated with a very high risk of HIV/Aids. These include:

- Anal penetrative sex without a condom.
- Unprotected anal contact if blood is present.

If this is a true description of your sexual behaviour, you are at an extremely high risk to contract HIV/Aids. A test to determine your HIV status is essential, as well as real changes to your sexual behaviour. Since there is more than one HI-virus, try to avoid "High Risk" and "Very High Risk" behaviour. With good nutrition and a healthy lifestyle, many people live with the virus for 15 years or longer.

[illegible]

Requirements achieved	YES	NO
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Comments in support of decision

QUESTION 9: Where can anyone obtain reliable HIV statistic.

- ♦ **SASETA HIV report**
- ♦ **The South African Department of Health Study, 2006**

Requirements achieved	YES		NO	
Comments in support of decision				

QUESTION 10: Explain the impact of HIV on the workplace.

Recruitment and training: The more skilled and unskilled employees you lose due to HIV/AIDS, the more time and money you need to spend on recruiting and training new staff.

Higher pension fund and insurance premiums: The more employees you lose due to HIV/AIDS, the more likely your premiums will increase.

Administration: As more of your employees become affected by or infected with HIV, the greater your administrative work. In some companies, a full-time person/team may be needed to handle the extra administrative work.

Medical care: As more of your employees become infected by HIV, your medical aid premiums will increase. If your company offers ill employees other kinds of healthcare support, your expenses in this regard will also climb.

Requirements achieved	YES		NO	
Comments in support of decision				

QUESTION 11: What effect have the employment equity Act on the management of HIV in the workplace.

- Force an employee or prospective employee to have an HIV Test without first getting permission from the Labour Court. Only under very specific circumstances, will the Labour Court give such permission.
- An employer cannot refuse to employ an HIV positive person, or dismiss an employee because of his/her HIV status.
- **An employee with HIV is not required to reveal his/her HIV status** to the employer. However, if an HIV positive employee does disclose his/her status, he/she may benefit if management is understanding and provides some form of help, treatment and support.
- If **an employer or co-worker becomes aware** of an employee's HIV status, **they are expected to keep such information confidential**, unless the employee with HIV gives specific consent for the information to be disclosed.
- **No employee may refuse to work with a fellow-employee with HIV**. Such refusal should lead to disciplinary action.

Requirements achieved	YES		NO	
Comments in support of decision				

QUESTION 12: What should you do to ensure that your company is legally compliant.

Requirements achieved	YES		NO	
Comments in support of decision				

QUESTION 13: List 5 types of management blocks that resulted in the delaying of HIV strategies.

When presenting your strategy, don't get put off if people block you with any of the following negative responses:

- They deny the fact that HIV/AIDS is a business threat by saying things like "We don't have a problem".
- They hear what you're saying but are apathetic or don't want to take any action and so will say things like "The problem is too big for us, so leave it alone" or "Let someone else deal with it, not us".
- They block you with a prejudiced attitude towards people living with HIV/AIDS by saying things like, "It's not our fault they have HIV".
- They use the excuse of time pressures by saying things like, "We're far too busy to worry about HIV/AIDS issues".
- They use the excuse of not having budget, people and other resources needed to implement HIV/AIDS related activities by saying things like "We don't have any money or the people to implement anything in the workplace".
- They use the excuse of not having sufficient information by saying things like "We don't know where to go to get support, so let's forget about it", etc.

Requirements achieved	YES		NO	
Comments in support of decision				

QUESTION 14: Explain the following methods to prevent HIV in the workplace:

- ♦ Peer education
- ♦ Voluntary counseling

Peer education

Often employees are more comfortable talking to their co-workers about HIV/AIDS related issues, and getting advice from them, rather than talking to their managers/supervisors.

Based on this, the idea of peer education is about using employees to educate, inform and advise their co-workers or peers on HIV/AIDS issues, provided they are correctly trained.

Voluntary counseling

Voluntary Counseling and Testing (VCT) which includes confidential HIV testing, together with pre- and post-test counseling is very important because:

- ♦ It is the only way in which your employees can know their status.
- ♦ It gives them an opportunity to think about their life style options and make appropriate choices.

Requirements achieved	YES		NO	
Comments in support of decision				

QUESTION 15: List 4 ways in which an employee can be supported during the event of HIV infection.

Treatment, care and support of your HIV positive employees, and possibly their families, can take various forms, this section gives you a number of different tools to consider. These include:

- Reviewing your company's medical aid benefits
- Helping employees access public health services
- Counseling
- Lifestyle and nutrition counseling and support
- On-site private healthcare services
- Post-exposure prophylactics
- Treatment of opportunistic infections
- Anti-retroviral therapy (ART)
- Incapacity management

Requirements achieved	YES		NO	
Comments in support of decision				
